

# ATHENS METROPOLITAN CLINIC

Department of Psychiatry  
Date of Evaluation: August 8, 2026

**Patient Name:** Sarah Jenkins  
**DOB / Age:** 14/05/1994 | 32 y.o.  
**Sex:** Female  
**MRN:** #993-41-A  
**Attending Physician:** Dr. A. Mercer, MD, Ph.D.

## CHIEF COMPLAINT (CC)

"I feel like I am constantly on edge and my heart won't stop racing."

## HISTORY OF PRESENT ILLNESS (HPI)

The patient is a 32-year-old female presenting with a 7-month history of excessive, uncontrollable worry pertaining to her career, finances, and the health of her family members. She reports accompanying somatic symptoms including muscle tension (predominantly in the cervical and trapezius regions), chronic fatigue, irritability, and sleep disturbances (difficulty initiating and maintaining sleep). She notes experiencing 2-3 episodes per month of acute autonomic arousal characterized by palpitations, diaphoresis, and a feeling of impending doom, lasting approximately 15 minutes.

## PAST PSYCHIATRIC HISTORY

No prior psychiatric hospitalizations. Intermittent therapy for "stress" during university (2014).

## SUBSTANCE USE

Denies illicit drug use. Alcohol: 1-2 glasses of wine per week. Caffeine: 4-5 cups of coffee daily (advised to reduce).

## MENTAL STATUS EXAMINATION (MSE)

Alert and oriented x3. Appearance is neat, appropriate for the weather. Psychomotor agitation noted (fidgeting with hands, restless legs). Speech is of normal rate and volume but slightly pressured when discussing work. Mood is "anxious." Affect is congruent, restricted, and tense. Thought process is linear and logical. Thought content reveals prominent themes of apprehension regarding future events; no auditory/visual hallucinations. Suicidal/homicidal ideation denied. Insight and judgment are good.

## ASSESSMENT

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- Generalized Anxiety Disorder (GAD) - F41.1
- Rule out Panic Disorder

## TREATMENT PLAN

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1. **Pharmacotherapy:** Initiate Escitalopram (Lexapro) 10 mg PO daily. Discussed potential side effects including GI upset and delayed onset of therapeutic action (4-6 weeks). Provided a short-term prescription of Lorazepam 0.5 mg PO PRN for severe panic attacks (max 10 tabs, no refills).
2. **Psychotherapy:** Referral placed for Cognitive Behavioral Therapy (CBT) focusing on anxiety management and cognitive restructuring.
3. **Lifestyle:** Strongly advised to decrease caffeine intake to <1 cup per day. Recommended mindfulness exercises and cardiovascular activity.
4. **Follow-up:** Scheduled in 4 weeks to assess medication tolerance and symptom progression.

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Dr. A. Mercer, MD, Ph.D.

Electronic Signature Validated